DISCHARGE SUMMARY

Patient details

Full name:	Robert Ellis (fictional)
Date of birth:	14 March 1957 (age 69)
MRN:	FICTIONAL-001
Ward:	Orthopaedic Ward 7B, Riverside Demo Hospital (fictional)
Admitted:	21 October 2026
Discharged:	24 October 2026
Primary contact:	Margaret Ellis (wife) - caregiver

Procedure and diagnosis

Procedure:	Left total hip arthroplasty (cemented)
Surgeon:	Dr. A. Nkosi (fictional), Orthopaedic Surgery
Anaesthetic:	Spinal anaesthesia with sedation
Diagnosis:	Primary osteoarthritis of the left hip, end-stage
Implant:	Details on file with the surgical team (not reproduced here)

Discharge status

Mr Ellis was discharged in a clinically stable condition following an uncomplicated left total hip arthroplasty. He is ambulatory with a walking frame. Oral analgesia has been commenced. All post-operative observations were within normal limits at the time of discharge. The patient was accompanied home by his wife.

Section 2 - Medications

2.1 Prescribed anti-inflammatory (pain management)

Mr Ellis has been prescribed an oral non-steroidal anti-inflammatory drug (NSAID) for post-operative pain management. The specific product and dose are documented on the pharmacy dispensing label attached to the medication packaging.

Dosing schedule: take as required when pain is present, OR on a regular scheduled basis as directed by the dispensing pharmacist.

Starting dose: adjust per individual tolerance and response. Do not exceed the maximum daily dose stated on the product label.

2.2 Paracetamol (adjunct analgesia)

Paracetamol 500 mg to 1 g may be taken as needed for additional pain relief. Frequency: every 4 to 6 hours as required.

2.3 Existing home medications

Continue ALL existing home medications unless otherwise instructed by the surgical team or GP.

2.4 Weaning off anti-inflammatories

The prescribed NSAID should be gradually reduced and discontinued as recovery progresses and pain subsides.

Section 3 - Wound Care

3.1 Initial dry period

The surgical wound must be kept completely dry for the first 48 hours following discharge.

3.2 Showering and bathing

The patient may take a light shower after day 3 post-discharge provided the wound area is adequately covered with a waterproof dressing.

Do not submerge the wound site in water (baths, swimming pools) until the wound is fully healed and the skin is intact.

3.3 Dressing changes

Perform daily dressing changes until the wound is fully healed. Dressing materials have been included in the discharge pack provided at the ward.

Do not disturb the primary wound dressing unless it becomes soiled or saturated. If in doubt, contact the district nursing team.

3.4 Wound monitoring

Inspect the wound at each dressing change. Contact the surgical team or attend the emergency department if any of the following are observed:

- Redness, increasing warmth or swelling around the wound
- Any discharge (clear, yellow or blood-stained)
- Wound edges separating or sutures/clips appearing loose
- Fever above 38.0 degrees C
- Increasing rather than decreasing pain after the first 48 hours

Section 4 - Mobility and Activity

4.1 Immediate post-discharge rest

Strict bed rest is recommended for the first 24 to 48 hours following discharge to allow the surgical site to stabilise.

4.2 Early mobilisation

Early mobilisation is strongly encouraged to reduce the risk of complications including deep vein thrombosis (DVT), chest infection and pressure injury. Short, gentle walks with the walking frame should begin as soon as the patient feels able to do so.

4.3 Hip precautions

Standard hip precautions must be maintained throughout the recovery period. Refer to the physiotherapy instruction sheet provided on the ward.

4.4 Weight bearing

Weight bearing: as tolerated, using the walking frame provided.

Note from physiotherapy assessment: protected weight bearing recommended until reviewed at follow-up. Do not progress beyond partial weight bearing without physiotherapist review.

4.5 Stairs

Stair use is permitted with caution once the patient has adequate strength and confidence. Always use the handrail and take one step at a time.

Avoid stairs where possible during the initial recovery phase.

4.6 Physical therapy

A physiotherapy referral has been submitted. Community physiotherapy will make contact to arrange an initial assessment. Begin gentle range-of-motion exercises when the patient feels comfortable and ready.

4.7 Driving

Mr Ellis must not drive until he is able to perform an emergency stop safely and has received medical clearance from his GP or the surgical team.

4.8 Return to work and domestic activities

Light domestic activities are permitted when the patient feels comfortable. Return to work should be discussed with the surgeon at the follow-up appointment. Heavy lifting (over approximately 5 kg) should be avoided until cleared by the surgical team - no specific timeframe is given.

Section 5 - Follow-up Appointments

5.1 Surgical follow-up

Attend a follow-up appointment at the Orthopaedic Outpatient Clinic in approximately 4 to 6 weeks post-discharge.

A written appointment confirmation will be sent by post to the patient's registered home address. No specific date or time was confirmed at the point of discharge. If the letter has not arrived within 10 days, contact the outpatient booking team at the number on your clinic letter.

5.2 GP review

Mr Ellis should see his GP within 2 weeks of discharge if he has any concerns or if recovery is not progressing as expected.

5.3 Blood tests

Routine blood tests are required at the follow-up appointment. Fasting may be required - await further instruction from the clinic.

Section 6 - Emergency Guidance

6.1 When to seek urgent help

Return IMMEDIATELY to the emergency department or call emergency services if experiencing any of the following:

- High fever (above 38.5 degrees C)
- Severe or rapidly worsening pain not responding to prescribed medication
- Sudden significant swelling of the operated limb
- Signs of wound infection (redness, heat, discharge, unpleasant odour)
- Calf pain and/or swelling in either leg (possible DVT)
- Shortness of breath or chest pain (possible pulmonary embolism)
- Dislocation or suspected dislocation of the hip implant

6.2 Non-urgent queries

For non-urgent post-discharge questions, contact the surgical team during working hours using the contact details provided on your discharge letter.

6.3 General welfare

If Mr Ellis appears generally unwell - such as confusion, unusual drowsiness, or significant deterioration in function - seek medical advice promptly.